

AI FOR BUSINESS TRACK · ROUND 2

Pavo Cloud

Autonomous Prior Authorization

The insurer's answer in 88.6 milliseconds. The whole round trip in under five minutes, not three to fourteen days.

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Prior Authorization Is Broken

3-14 days

to a decision

94%

of doctors report delays

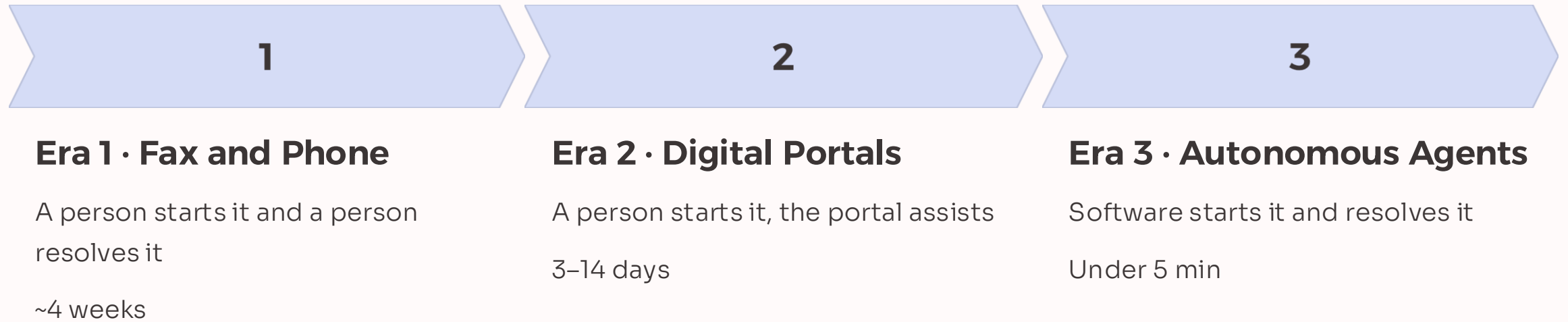
97%

of requests approved

\$35B

spent on the process each year

The Three Eras of Prior Authorization



Era 3: Agent to Agent

SOLUTION



Submitted Automatically

Clinical documentation leaves the EHR directly. No staff touchpoint, no portal login.



Unclear Cases Escalate, Assembled

A human reviewer opens a file already complete: records, rule, signature check, confidence score.



Clear Cases Close in Milliseconds

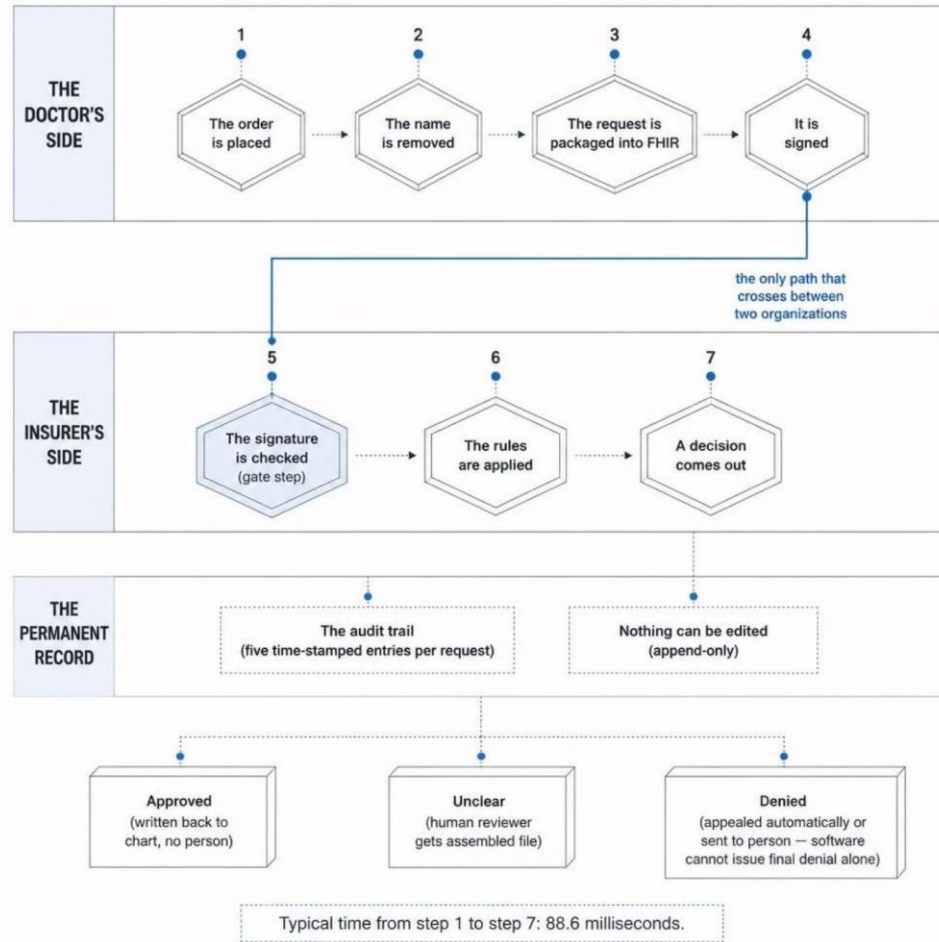
The full decision path takes 88.6ms, and 84.0 of those are the two RSA-2048 signatures every authorization writes. The deciding itself is the cheap part.



The Doctor Stays in Charge

Medical necessity remains a physician's call. Only the administrative burden disappears.

System Architecture



How It Works -

Stack: FastAPI · Next.js 14 · Supabase-ready · FHIR R4 · RSA-2048 signing · ZK circuits (circom/snarkjs) · timestamped audit log

Speed: Step 1 to step 7 in 88.6ms (median, n=200)

TRACTION

Customer Discovery & Validation

12+

interviews conducted with hospital administrators and clinical staff

8

insurance provider conversations completed

3

test surveys distributed to prior auth teams

2

letters of interest received from health systems

100%

of interviewed providers cited PA delays as a top operational pain point

The Safe Outcome Is the Default

Fixed Rules – Decides

Same input, same answer, every time

- Identity checks
- Coverage rules
- Price arithmetic
- Denial-reason sorting

AI Models – Assist Only

Every result carries a score. Below a threshold → a person

- Reading insurance cards
- Mapping plain English to a medical code
- Drafting appeal letters

Unmatched request → human reviewer, within 4 hours

Appeal below 70% confidence → held for a person

AI provider offline → authorization is unaffected

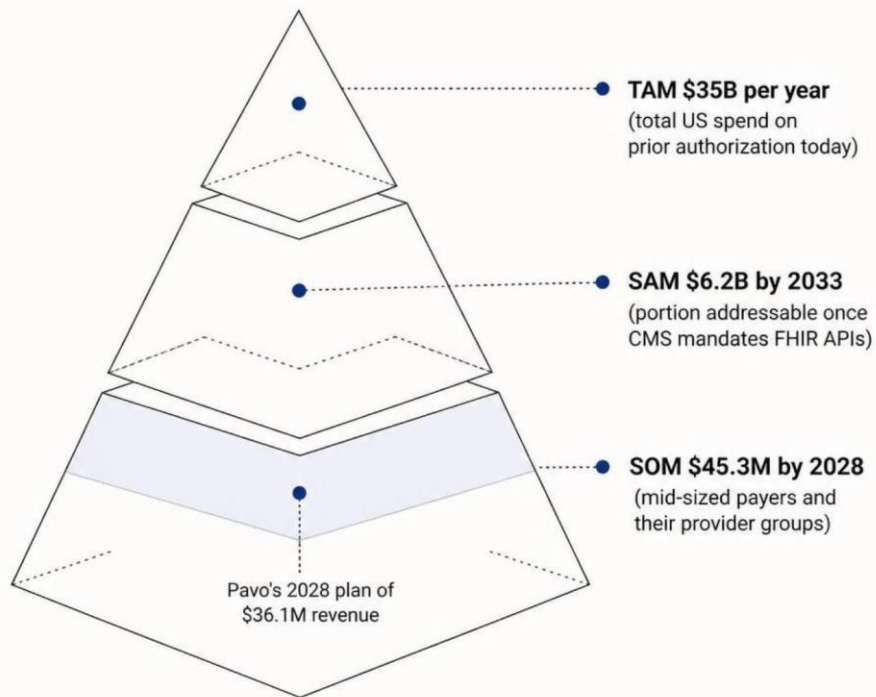
What We Have Not Solved

Gap	What it needs
The privacy proof reveals which covered condition a patient has	A more advanced circuit. Scoped for the next phase.
The proof's setup was done on one machine	A proper multi-party ceremony before any real use.
Provider identity is trusted, not verified	One integration against the federal provider registry.
Five coverage rules, not a rule library	Clinical staff encoding each insurer's criteria: our largest cost, and the real barrier to a competitor.

Everyone Else Still Waits for a Person

Capability	Pavo Cloud	Cohere Health	Availity AuthAI
Software can start a request with no human	✓	—	—
Decision without a human in the loop for clear cases	✓	—	—
Cryptographic sender verification	✓	partial	partial
Zero-knowledge patient privacy proof	✓	—	—

A Market a Regulation Is About to Create



Why Now

Federal mandate: CMS-0057-F requires FHIR-based prior-auth APIs by January 1, 2027, every payer has a hard deadline.

AI inflection: Agents now need a rail to initiate authorizations. Pavo is that rail.

Scale already exists: 47M payer-provider interactions a year still move through portals and fax.

How Pavo Makes Money

BUSINESS MODEL

\$3.00 per request, charged to the insurer	\$400 per month, per medical practice	\$20,000 per year, per insurer connection	
Source	2026	2027	2028
Medical practices served (average)	55	330	1,400
Requests processed	171,600	1,647,360	8,736,000
Fees per request, at \$3.00	\$514,800	\$4,942,080	\$26,208,000
Practice subscriptions, at \$400/month	\$264,000	\$1,584,000	\$6,720,000
Insurer connection fees, at \$20,000/year	\$220,000	\$1,680,000	\$3,200,000
Total revenue	\$998,800	\$8,206,080	\$36,128,000

The \$3.00 fee is 85-92% below the \$15-\$40 an insurer spends handling a request by hand today, which is what makes the switch easy to justify.

What It Costs, and What Is Left

MARGIN

31%

gross margin, 2026

82%

gross margin, 2027

87%

gross margin, 2028

Servers are not the expense

Processing one request costs about six hundredths of a cent, because the deciding path uses no AI models at all.

Clinical staff are

Translating each insurer's published rules into the fixed rules the software applies is the largest line in every year, and the reason margins improve with scale rather than with technology.

Year 1 is 31%, not break-even. Early customers need a great deal of hand-holding, and we would rather say so.

Cost to Win a Customer, and What One Is Worth

UNIT ECONOMICS

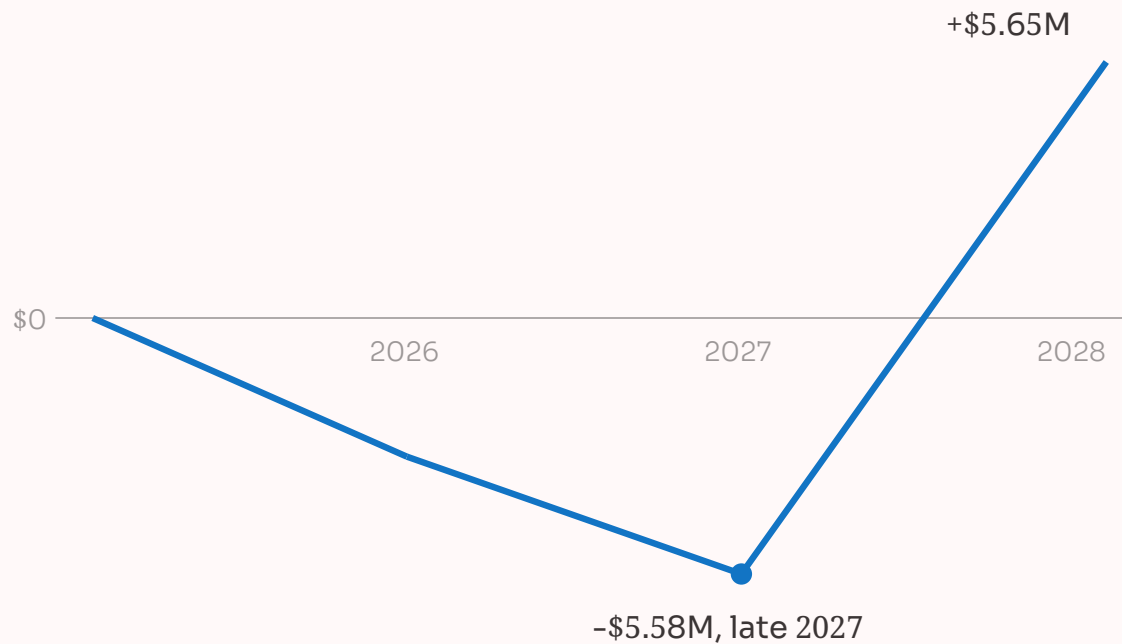
Acquisition cost is what it costs in sales and marketing to sign one customer. Lifetime value is the profit that customer produces before they leave, stated here over three years, which is deliberately conservative.

Measure	Practice	Insurer	Blended
Cost to acquire one customer	\$6,000	\$22,000	\$6,699
Value over three years	\$58,248	\$51,057	\$57,533
Value per \$1 spent acquiring	\$9.70	\$2.30	\$8.60
Months to earn the cost back	3.5	15.2	4.0

Read the insurer column honestly. At \$2.30 back per \$1 spent and fifteen months to recover it, insurer contracts do not pay for themselves as a product. We fund them anyway, because signing one insurer makes Pavo available to every practice that already submits to it. It is a distribution channel, not a profit centre.

We Are Asking For \$8.0 Million

THE ASK



Cumulative cash. The raise covers the trough with roughly twelve months of cushion past breaking even.

The clinical rule library

Our largest cost and the real barrier to a competitor.

Security certification

No insurer signs without it.

Closing the gaps we listed

Provider registry, multi-party ceremony, rule library.

It is not a shopping list. The three largest spending lines across the plan, \$7.46M of engineering, \$5.67M of insurer business development and \$1.72M of clinical staff encoding coverage rules, come to more than the raise on their own, and most of that is paid for out of revenue as it arrives.

If all four of our main assumptions are wrong at once, 2028 revenue is \$13.8 million rather than \$36.1 million: a smaller company, but still a real one.